

MSK:Upper Extremity Physical Exam

Diagnostic Medicine III 2025

STARTING THE PATIENT ENCOUNTER

Introduction, Verify Patient

General Survey

Vital Signs (obtain or repeat abnormal vitals)

UPPER EXTREMITY

INSPECTION Anteriorly & Posteriorly

(Verbalize)

Abnormal positioning

Deformity

Swelling

Fasciculations

Muscular atrophy

SAM DF

SHOULDER

PALPATION BILATERALLY

Sternoclavicular joint

Clavicle

Acromioclavicular joint

Coracoid process (beneath clavicle and anterior to acromion)

Greater tubercle (2 inch above IM inj site)

Biceps tendon at bicipital groove (2 inch above armpit)

Subacromial & subdeltoid bursae (above greater tubercle - tip of shoulder) lift patient's elbow posteriorly

Shoulder muscles (SITS-supraspinatus, infraspinatus, teres minor, subscapularis) 1. Find spine of scapula 2. Roll fingers above to separate deltoid and find subscapularis

Verbalize: palpating for soft tissue, bony or joint tenderness, warmth, swelling, crepitus, and masses

ASSESSES: Active ROM (AROM) & Passive ROM

Bilaterally

Flexion/Extension Raise arm forward over head/pull arm backwards

Abduction/Adduction pull arm to side

Internal/External Rotation Hold elbow steady and "show me how big fish was" (external)

SHOULDER Special Tests

Empty Can test (supraspinatus)- patient elevates arms to 90 degrees and internally rotates the arms with the thumbs pointed down. Ask patient to resist as you apply downward pressure to the forearms



Infraspinatus test- Patient holds arm at their side with elbow flexed to 90 degrees and then externally rotates shoulder against resistance



Subscapularis test-patient holds arm at their side with elbow flexed to 90 degrees and then internally rotates shoulder against resistance



Crossed body adduction test (acromioclavicular joint)- Adduct the patient's arm across the chest.



Neer test (subacromial impingement)-Forward flex patient's arm while depressing scapula or acromion



Hawkins test (subacromial impingement)-Abduct patient's shoulder to 90 degrees with elbow flexed at 90 degrees and internally rotate shoulder



ELBOW

PALPATION BILATERALLY

Olecranon process

Radial head

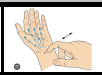
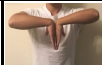
Biceps brachii tendon

Medial epicondyle

Lateral epicondyle

Verbalize: palpating for soft tissue, bony or joint tenderness, warmth, swelling, crepitus, and masses or nodules



ASSESESSES: Active ROM (AROM) & Passive ROM Bilaterally	Forward Flexion and Extension	
	Forearm Supination /Pronation	
SPECIAL TESTS	Forearm Supination/Pronation against resistance medial epicondylitis lateral epicondylitis	
WRIST, HANDS, FINGERS, THUMB		
PALPATION BILATERALLY	Distal Radius	
	Distal ulna	
	Anatomic snuffbox	
	Carpals	
	Metacarpals	
	MP Joint	
	PIP Joint	
	DIP Joint	
	Palm Verbalize: palpating for thickening and contractures.	
	Verbalize: Palpating for soft tissue, bony or joint tenderness, warmth, swelling, boggiess, crepitus, and masses	
ASSESESSES: Active ROM (AROM) & Passive ROM Bilaterally "copy my movements"	Wrist Flexion /Extension	
	Wrist Ulnar/Radial Deviation	
	Finger Flexion/Extension	
	Finger Abduction /Adduction	
	Thumb Flexion/Extension	
	Thumb Adduction/Abduction	
	Thumb Opposition	
SPECIAL TESTS		
Tinel sign (for Carpal tunnel syndrome)-Examiner taps volar aspect of wrist between thenar and hypothenar proximal eminences		
Phalen test (for Carpal tunnel syndrome)-Patient holds wrists fully flexed with dorsal surfaces held together for 1 minute		
Hand grip strength- Ask the patient to grasp your second and third fingers as tightly as possible. Ulnar nerve test		
Thumb tenosynovitis (Finkelstein) test- Patient grasps the thumb against the palm and then moves the wrist toward the midline in ulnar deviation. Complete bilaterally for campison. Positive sign- pain with ulnar deviation		
Thumb Abduction test - patient abducts thumb against resistance Carpal Tunnel		
Consider whether a neurological examination is necessary when completing a musculoskeletal examination.		